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Supplement Table 2. Baseline characteristics of included studies

Study	Inclusion criteria	Age (y) All participants (experimental/ comparator) Comorbidity	Baseline IIEF All participants (experimental/ comparator)	Participant's disposition (randomized/ analyzed/ completed the trial)	Experimental intervention(s) (dose) No of participants, randomized /analyzed Durationof intervention	Comparator (s) (no. of participants, randomized/ analyzed)	Outcome measures	Study design Trial period (year to year) Setting (Country)	Funding Support for ginseng for experiment
Choi et al [11], 1995	- ED classified as type I and type IIb by radioisotope AVS-penogram - ED without organic dysfunction	- NR (KRG, 42.8/ placebo, 45.2; trazodon, 43.2) - NR	IIEF was not used but measur ed with RAVS-penogram - NR (NR/NR)	90/90/90	- KRG (1,800 mg) - 30/30 - 12 weeks	C1: placebo (30/30) C2: trazodone. 25 mg (30/30)	1) AEs 2) Participant's ability to have intercourse reported by participant (or partner) (self-reported improvement in erection-not validated questionnaire)	- Parallel - 1994–1994 - Urology clinic in 1 university hospital (S.Korea)	- KT&G - Ginseng seemed to be supported by KT&G but NR
Choi et al [8], 1999	- Penile rigidity under 70% on the audiovisual sexual stimulation test under psychogenic ED - Mild or moderate organic ED - ED from unknown cause or without organic dysfunction	- NR (KRG: Korea, 43.4, China, 39.1, Singapore, 50.2/ placebo: Korea, 45.2, China, 42.9, Singapore, 43.9) - Diabetes (n=7), hypertension (n=7), hypercholesterolemia (n=9)	IIEF was not used but measur ed with RAVS-penogram - NR (NR/NR)	70/64/64	- KRG (1,800 mg) - 40/37 - 12 weeks	Placebo (30/27)	1) AEs 2) Participant's ability to have intercourse reported by participant (or partner) (self-reported improvement in erection-not validated questionnaire)	- Parallel - NR - Urology clinics in 2 university hospitals and andrology clinic in 1 local hospital (S.Korea, China, and Singapore)	- NR - Ginseng was supported by KT&G
Choi and Choi [9], 2001	- Over 20 years old - Clinical ED without definite organic cause	- 45.7 (46.1/45.4) - No comorbidities	- 13.02 (19.82/ 14.40) in IIEF-EF (moderate or mild to modera te ED)	50/47/47	- KRG (1,800 mg) - 25/24 - 8 weeks	Placebo (25/23)	1) EF (IIEF-15) 2) AEs	- Parallel - NR - Urology clinic in 1 university hospital (S.Korea)	- NR - Ginseng seemed to be supported by KT&G but NR

Supplement Table 2. Continued 1

Study	Inclusion criteria	Age (y) All participants (experimental/ comparator) Comorbidity	Baseline IIEF All participants (experimental/ comparator)	Participant's disposition (randomized/ analyzed/ completed the trial)	Experimental intervention(s) (dose) No of participants, randomized /analyzed Durationof intervention	Comparator (s) (no. of participants, randomized/ analyzed)	Outcome measures	Study design Trial period (year to year) Setting (Country)	Funding Support for ginseng for experiment
Choi et al [10], 2003	- Over 20 years old - Clinical ED without definite organic cause	- 44.5 (45.1/44.4) - NR	IIEF was measured but not reported - NR (NR/NR)	30/28/28	- KRG (1,800 mg) - 20/19 - 4 weeks	Placebo (10/9)	1) AEs 2) Participant's ability to have intercourse reported by participant (or partner) (GAQ score)	- Parallel - NR - Urology clinic in 1 university hospital (S.Korea)	- KT&G - Ginseng seemed to be supported by KT&G but NR
de Andrade et al [12], 2007	- 34 to 67 years old - IIEF-5 scores between 13 and 21 (mild or mild to moderate ED)	- NR (52.6/54.3) - Diabetes (n=10), hypertension (n=22), cardiovascular disease (n=5)	- NR (16.4/17.0) (mild or mild to moderate ED)	60/60/60	- KRG (3,000 mg) - 30/30 - 12 weeks	Placebo (30/30)	1) EF (IIEF-5) 2) AEs 3) Participant's ability to have intercourse reported by participant (or partner) (GAQ)	- Parallel - 2004–2004 - Urology clinic in 1 university hospital (Brazil)	- NR - NR
Ham et al [13], 2009	- Over 11 points of IIEF scores - ED for more than 3 months	- NR (53.2/50.8) - Diabetes (n=15), hypertension (n=17)	- NR (17.2/17.7) (moderate or mild to moderate ED) in IIEF-EF	73/69/69	- KRG Plus ginsenoside (800 mg) - 37/35 - 8 weeks	Placebo (36/34)	1) EF (IIEF-15) 2) AEs	- Parallel - 2007–2007 - Urology clinic in 2 university hospitals (S.Korea)	- BT Gin Inc., MoHK - Ginseng was supported by BT Gin
Hong et al [14], 2002	- ED without definite organic cause	- 54 (NR/NR) - Diabetes (n=8), hypertension (n=15), abnormal total serum cholesterol (n=5), cerebrovascula r disease (n=3), pulmonary disease (n=5), liver disease (n=1), BPH (n=5), history of surgery for rectal cancer (n=6)	- 8.93 (8.93/8. 93) (moderate) in IIEF-5	45/90/90	- KRG (2,700 mg) - 22/45 - 8 weeks	Placebo (23/45)	1) EF (IIEF-5, IIEF-15) 2) AEs 3) Participant's ability to have intercourse reported by participant (or partner) (GAQ)	- Cross-over - NR - Urology clinic in 1 university hospital (S.Korea)	- NR - Ginseng was supported by KT&G

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Supplement Table 2. Continued 2

Study	Inclusion criteria	Age (y) All participants (experimental/ comparator) Comorbidity	Baseline IIEF All participants (experimental/ comparator)	Participant's disposition (randomized/ analyzed/ completed the trial)	Experimental intervention(s) (dose) No of participants, randomized /analyzed Durationof intervention	Comparator (s) (no. of participants, randomized/ analyzed)	Outcome measures	Study design Trial period (year to year) Setting (Country)	Funding Support for ginseng for experiment
Kim and Paick [15], 1999	Mild vasculogenic impotence (absence of full rigidity on the pharmacologic erection test Peak systolic velocity in the cavernous arteries of 20 to 35 cm/sec)	NR (45.6/44.8) NR	NR (25.7/2 6.3) measured with Watts Q	26/21/21	- KRG (2,700 mg) - 13/11 - 12 weeks	Placebo (13/10)	1) EF (modified Watts Q)	- Parallel - NR - Andrology clinic in 1 university hospital (S.Korea)	- KT&G - Ginseng was supported by KT&G
Kim et al [16], 2009	Under 51 points in the total IIEF score No allergy to ginseng no acute illness	NR (57.5/ 60.2) Diabetes mellitus (n=19), hypertension (n=15), hyperlipidaemia (n=16)	NR (11.89/ 11.38) (moderate) in IIEF-EF	143/86/86	- TCMG (2,000 mg) - 75/65 - 8 weeks	Placebo (68/21)	1) EF (IIEF-5 and IIEF-15)	- Parallel - NR - Outpatient department in 1 university hospital (S.Korea)	Kyunghee Univ. Two authors were affiliated with MG production institute

AEs: adverse events, AVS: audio-visual stimulation, C: comparator, ED: erectile dysfunction, EF: erectile function, GAQ: Global Assessment Questionairre, I: intervention, IIEF: International Index of Erectile Function, KRG: Korean red ginseng, MoHK: Ministry of Health, South Korea, NR: not reported, RAVS: radioisotope audio-visual stimulation, TCMG: Tissue-cultured mountain ginseng, Watts Q: Watts Sexual Function Questionnaire.

Note: 1) No studies did the follow-up for outcomes. 2) The severity of ED with IIEF-15 was classified into five categories: no ED (EF score 26–30), mild (EF score 22–25), mild to moderate (EF score 17–21), moderate (EF score 11–16), and severe (EF score 6–10). 3) The possible scores for the IIEF-5 range from 5 to 25, and ED was classified into five categories based on the scores: severe (5–7), moderate (8–11), mild to moderate (12–16), mild (17–21), and no ED (22–25).